

CLINICAL INTELLIGENCE TREND REPORT

YELLOW: Closer Observation Suggested

Patient ID: JBPatient ID: JBPeriod: June 23 to July 23, 2024Report Date: July 24, 2024Coverage: Bed: 609/744h (81.9%)Chair: 502/744h (67.5%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm

Patient clinical status over 31 days from Jun 23 to Jul 23

HR

Breathing



Last 24 Hours

Last 24 Hours: ELEVATED

30-Day Tier: YELLOW

Vital	Avg	Min	Max
Heart Rate	57 bpm	40 bpm	86 bpm
Breathing	14 brpm	9 brpm	30 brpm

Last 24 hours: three low heart rate episodes overnight.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Jul 23	12 AM to 4 AM	7h	3	Elevated Breathing	14	9	30	Check SpO2 and lung sounds; assess for fluid overload or early infection
Jul 23	12 AM to 4 AM	7h	3	Low Heart Rate (brief)	42	40	69	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic

115 episodic events Detected, spanning 7 days total. Conditions: Elevated Breathing, Elevated Heart Rate, High Breathing, High Heart Rate, Low Heart Rate, Very High Heart Rate. **Priority grade: Medium**

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
115	183h	Elevated Heart Rate	4	100%

Major Findings: Sustained elevated HR (avg 98, peak 131)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Jul 08	12 AM to 1 AM	7h	6	High Heart Rate	104	45	127	Check temp, hydration, pain; review for arrhythmia or infection
Jun 30	7 AM to 8 AM	7h	7	Low Heart Rate	44	40	60	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Jul 13	10 PM to 11 PM	61h	26	Low Heart Rate	43	40	74	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Jun 23	12 AM to 1 AM	77h	53	Elevated Heart Rate	98	46	131	Check temp, hydration, pain; review for arrhythmia or infection
Jun 23	7 AM to 8 AM	10h	9	Elevated Breathing	28	13	48	Check SpO2 and lung sounds; assess for fluid overload or early infection
Jul 15	7 PM to 8 PM	21h	14	Elevated Breathing	26	9	36	Check SpO2 and lung sounds; assess for fluid overload or early infection
Jun 29	2 AM to 3 AM	6h	5	Very High Heart Rate (brief)	112	83	130	Check temp, hydration, pain; review for arrhythmia or infection

+ 1 additional condition(s) across High Breathing (brief); details in monitoring data.

The P5 to P95 heart rate spread was 52 bpm (44 to 96), indicating significant cardiac variability.

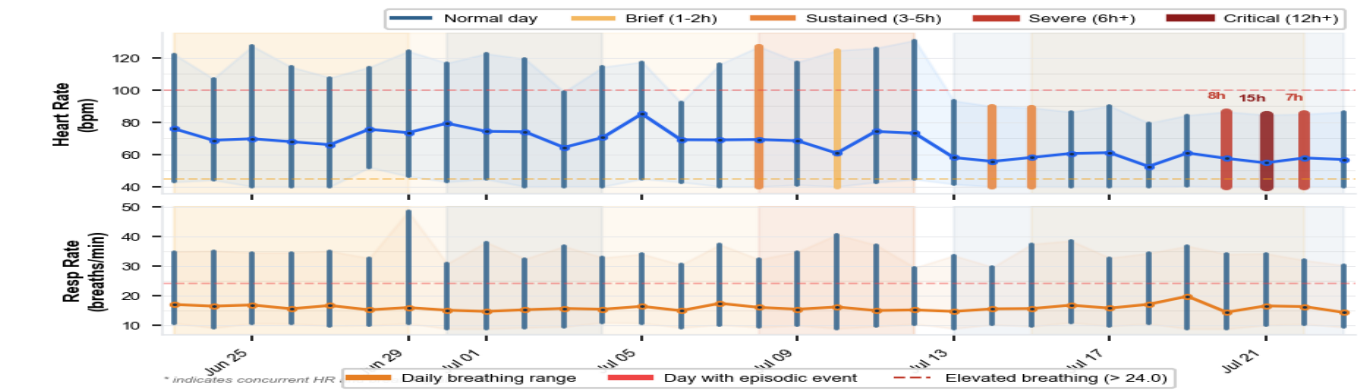
Suggested Clinical Review Actions

- Elevated Heart Rate (avg 98 bpm, peak 131 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Low Heart Rate (avg 43 bpm, min 40 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Elevated Breathing (avg 26 brpm, peak 36 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection

- High Breathing (avg 33 bpm, peak 37 bpm): Check SpO₂, work of breathing, fever; consider pulmonary cause or CHF
- High Heart Rate (avg 104 bpm, peak 127 bpm): Check temp, hydration, pain; review for arrhythmia or infection

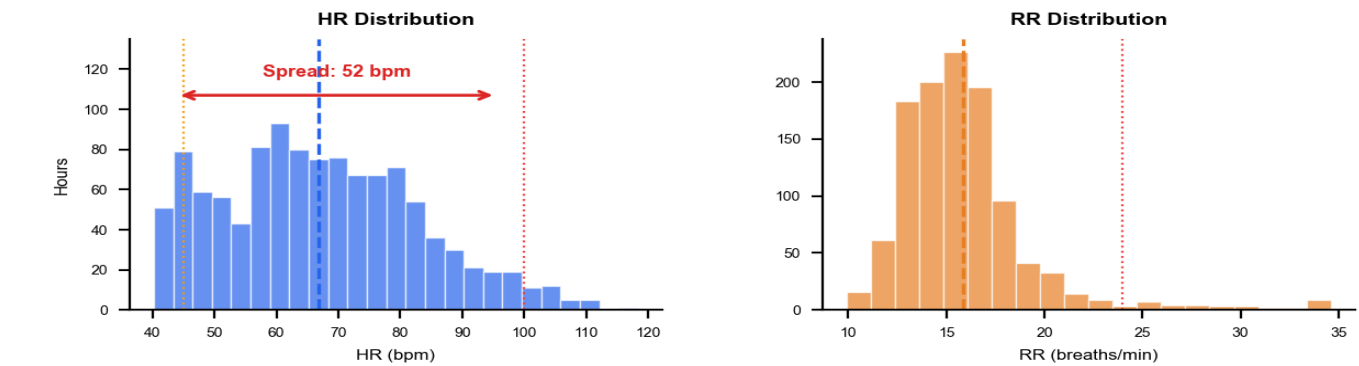
Clinical Pattern Observations:

- 1. **Sustained finding:** 8h continuous Low Heart Rate beginning Jul 20.
- 2. **High variability:** HR spread 52 bpm (43 to 95).
- 3. **Clustered pattern:** Episodic events on 7 of 31 days (22%).

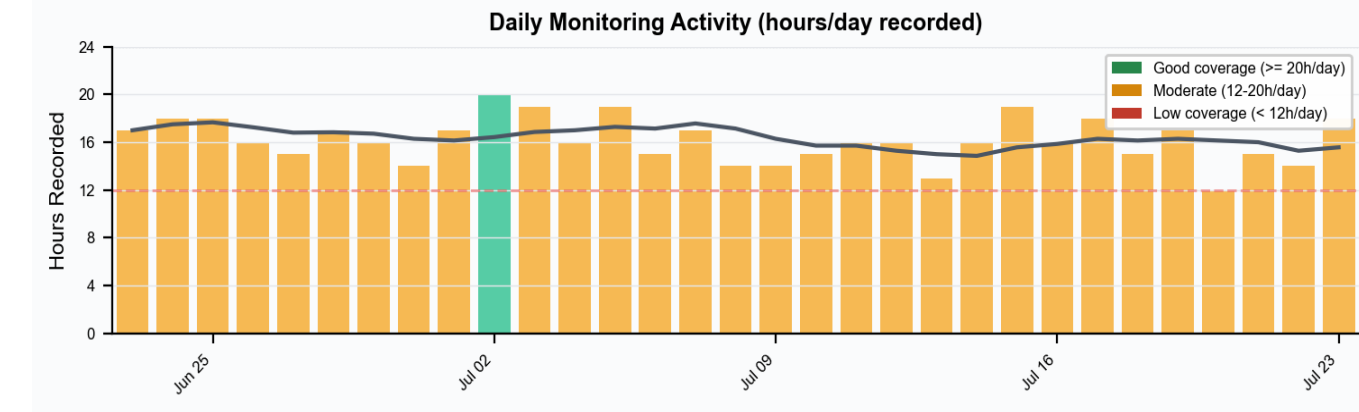


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.